Lancaster University

MANUAL OF ACADEMIC REGULATIONS

AND PROCEDURES

MEDICAL DEGREE ASSESSMENT REGULATIONS

(APPLICABLE FROM SEPTEMBER 2026)

Academic Quality, Standards and Conduct

Student and Education Services

Applicable from: September 2026

Planned Review Date: September 2028

CONTENTS

MD 1 STRUCTURE OF THE MBChB DEGREE PROGRAMME 2

MD 2 CRITERIA FOR AWARD 3

MD 3 PENALTY FOR LATE SUBMISSION OF COURSEWORK..... 4

MD 4 PROGRESSION..... 5

MD 5 CLASSIFICATION OF AWARDS..... 7

MD 6 REASSESSMENT..... 9

MD 7 INCOMPLETE ASSESSMENT AND EXCEPTIONAL CIRCUMSTANCES 10

MD 8 CONSIDERATION AND CONFIRMATION OF RESULTS 13

MD 9 PUBLISHED INFORMATION..... 16

MD 10 EXCLUSION..... 16

APPENDIX 1: HISTORIC PROGRESSION REQUIRMENTS 17

APPENDIX 2: HISTORIC TRANSITIONAL AGREEMENT FOR THE MLA..... 19

APPENDIX 3: EXIT AWARDS FOR THE MBCHB PROGRAMME 20

APPENDIX 4: SUMMARY OF CHANGES FROM 2025/26 MARP 22

APPENDIX 5: DETAIL OF IN-YEAR CHANGE MADE TO CHAPTER 26

ASSESSMENT REGULATIONS FOR THE BACHELOR OF MEDICINE, BACHELOR OF SURGERY (MBChB) UNDERGRADUATE DEGREE

The MBChB programme is under continuous review both internally and by the General Medical Council. As a result, it may sometimes be necessary to amend these regulations over the course of a student's registration to match national decisions about undergraduate medical education and to ensure continued alignment with sector standards. Changes will be made only where necessary to ensure the appropriate level of competence in graduates from the programme as they move onto their postgraduate training, and to ensure patient safety.

MD 1 STRUCTURE OF THE MBChB DEGREE PROGRAMME

MD 1.1 The MBChB degree comprises learning across levels 4, 5, 6 and 7 over a five-year full-time programme. There is no stipulated credit length for each year of study as medical qualifications are not typically credit rated.

MD 1.2 The Medicine with a Gateway Year degree is a six-year programme comprised of an initial year of study (Gateway Year), with progression onto the five-year MBChB degree programme where students meet the progression criteria.

MD 1.3 Fitness to practise requirements apply to students studying on the Gateway Year as well as to those on the MBChB degree programme. Fitness to practise requirements apply to all course content, regardless of whether this provision is delivered directly by the Lancaster Medical School, or by another academic department or source.

MD 1.4 Attendance and engagement in scheduled teaching, clinical placements and other prescribed course activities are mandatory requirements of the MBChB programme and support achievement of the General Medical Council's Outcomes for Graduates.

Students must satisfy the published attendance, engagement and professional requirements for the relevant year of study, as determined through the Medical School's approved monitoring processes.

Where a student does not satisfy these requirements, and no approved exceptional circumstances or other authorised absence applies, the Examination Board may determine that the progression requirements for the relevant year have not been met.

MD 1.5 Lancaster Medical School is committed to the principles of inclusive assessment and anticipatory reasonable adjustment. To support equity and accessibility, all timed, invigilated written examinations organised by Lancaster Medical School include an additional 25% time.

The published duration of Lancaster Medical School written examinations includes this universal adjustment and represents the standard examination duration for all students.

Students with an Inclusive Learning Support Plan (ILSP) or other approved reasonable adjustment are not automatically entitled to an additional 25% examination time beyond the published examination duration. Where a student requires adjustments

beyond those already incorporated within the assessment design, these will be considered through the University's disability and inclusion processes.

The inclusive adjustment described above applies only to Lancaster Medical School written examinations. It does not automatically apply to externally set or externally regulated assessments, including the Medical Schools Council Assessment Alliance Applied Knowledge Test or the Prescribing Safety Assessment. Any adjustments for such assessments will be determined in accordance with the requirements of the relevant assessment provider and University procedures.

This inclusive adjustment does not apply to coursework or other untimed assessments, for which reasonable adjustments will be considered through the University's approved disability support arrangements.

MD 1.6 The arrangements for the MBChB programme, including the programme structure, curriculum and associated requirements, shall be published in the relevant programme documentation.

MD 2 CRITERIA FOR AWARD

MD 2.1 In order to qualify for the MBChB degree, students must attain a pass in all assessment components of each of the five years of the programme. No compensation of failed assessment components is permitted. The standard-setting procedures described below are undertaken in accordance with the General Medical Council's requirements.

MD 2.2 The pass mark for each summative component of the assessment of each year of the MBChB degree will be arrived at as follows.

(a) Written examinations in Years 1–4: each examination paper will be standard set using an agreed standard-setting method, normally the modified Angoff method. The initial pass mark will be determined by the standard-setting panel without prior knowledge of student performance. The pass mark may subsequently be reviewed in light of additional information, including item analysis and student feedback, in accordance with the agreed standard-setting methodology.

(b) For Year 3, written assessment is organised across two assessment windows and multiple clinical science domains. Each written paper will be standard set using an agreed standard-setting approach, normally the modified Angoff method. Student performance will be interpreted by clinical domain, by assessment window and across the combined Year 3 clinical science evidence profile.

For the purposes of Year 3 progression decisions, written assessment outcomes will be classified as passed, borderline or significant evidence deficit. A borderline outcome is defined as performance below the standard-set pass mark but within two standard errors of measurement of that standard. Significant evidence deficit is defined as performance more than two standard errors of measurement below the standard-set pass mark.

The use of standard error of measurement thresholds is intended to support defensible interpretation of assessment evidence and does not constitute compensation between assessment components.

(c) Practical examinations (Objective Structured Clinical Examinations (OSCEs)) in Years 2–4 will usually be standard set using borderline regression. The overall cut score is the sum of the pass marks for each station within the examination. Where the standard OSCE comprises fourteen assessed stations, the minimum station-pass requirement will normally be ten stations.

(d) In Year 3, the OSCE is delivered across two assessment windows. The Year 3 OSCE standard will require both achieving the aggregate OSCE cut score and passing the required minimum number of stations across the combined Year 3 OSCE evidence profile. Where the standard Year 3 OSCE comprises fourteen assessed stations, the minimum station-pass requirement will normally be ten stations.

Where a student is required to undertake a targeted reassessment OSCE, this comprises seven stations. The combined station-pass requirement will normally be 15 passed stations out of 21, alongside any aggregate standard determined for the combined OSCE evidence profile.

Where a student has not had a valid OSCE sitting, any subsequent OSCE may be treated as a replacement assessment rather than a targeted reassessment.

(e) Coursework, including Special Study Module (SSM) coursework, will usually be marked to a standardised rubric. Moderation is undertaken in line with Lancaster University's General Regulations for Assessment and Award.

(f) The component clinical placement rotations of year 5 are assessed through the student's portfolio, on a pass/fail basis; each rotation must be passed. The clinical supervisor for the rotation assesses the student using the PETA process (Professional Education and Training Appraisal); signing the final PETA form as a pass indicates that the student has attended and has acquired the appropriate skills, knowledge, and professional attitudes to proceed into their Foundation Doctor years.

(g) The 'Preparing for Practice' programme in Year 5 must also be passed. The two-week 'Preparing for Practice' component at the start of the year is assessed through completion of a logbook of clinical skills and medicines management and must be passed.

MD 3 PENALTY FOR LATE SUBMISSION OF COURSEWORK

MD 3.1 Work submitted up to three days late without an agreed extension will receive a deduction of one grade and one ranking point.

MD 3.2 For work submitted up to three days late, where the application of the penalty would take the mark below the grade of 'pass', the penalised mark will be recorded as 'pass',

and a score of 0 ranking points, with no requirement to resubmit. Work received after three days will receive a mark of zero (non-submission). Saturdays and Sundays are included as days in this regulation; however, where the third day falls on a Saturday or Sunday, students will have until 10.00 a.m. on Monday to hand in without receiving further penalty.

MD 3.3 Work submitted more than three days late without an agreed extension will not be marked and the student will receive a classification of 'fail' and a score of 0 and be awarded 0 ranking points.

MD 4 PROGRESSION

MD 4.1 Progression from the initial (Gateway) year of the Medicine with a Gateway Year degree is dependent on attainment of the following:

(a) an aggregation score of at least 15.0 in all three units; and

(b) an aggregation score of at least 12.0 in three out of four end-of-year examinations.

All modules are non-compensable.

Examination Boards will determine whether a student has successfully met the progression requirements to move to the Year 1 MBChB, giving full countenance to exceptional circumstances as reported from the Exceptional Circumstances Committee and reassessment opportunities as detailed below.

Where these progression criteria are not met, students may be eligible for transfer into the second year of specified Biomedical and Life Sciences degree programmes, provided they meet the progression criteria set out for these programmes.

MD 4.2 The progression requirements of the MBChB degree have been detailed and approved through the programme approval process and are as follows:

Progression	Required to pass with the pass marks as detailed above
Year 1 to 2	Paper 1; Paper 2; Paper 3; Study Skills Module (completion of formative tasks); completion of the Community (Primary Care) course. Satisfactory attendance and engagement in accordance with MD1.4.
Year 2 to 3	Paper 1; Paper 2; Paper 3; SSM2; OSCE; hospital placement attendance and completion of the Community (Primary Care) course; completion of the E-Portfolio. Satisfactory attendance and engagement in accordance with MD1.4.
Year 3 to 4	Satisfactory Year 3 clinical science evidence across the required written assessment windows and clinical domains; satisfactory Year 3 clinical skills evidence through the OSCE assessment profile; pass in Medical Ethics and Law; pass in Epidemiology and Evidence-Based Medicine; completion of required workplace-based and portfolio evidence; hospital and GP placement attendance; completion of the E-Portfolio; satisfactory attendance in accordance with MD 1.4.

Year 4 to 5	Medical Licensing Assessment Applied Knowledge Test (MLA-AKT); Medical Licensing Assessment Clinical and Professional Skills Assessment (MLA-CPSA); hospital and GP placements attendance; completion of the E-Portfolio. Satisfactory attendance and engagement in accordance with MD1.4.
Graduation	Pass in all components of the 'Preparing for Practice' programme. Pass in the PETA (Professional Education and Training Appraisal) assessment of the five placement blocks (Ward Care; Acute Care-Emergency Medicine; Selectives in Advanced Medical Practice 1 and 2; Community-Orientated Medical Practice). Elective supervisor form completed. Completion of the E-Portfolio. Satisfactory attendance and engagement in accordance with MD1.4.

Examination Boards will determine whether a student has successfully met the progression requirements to move to the next year of the programme and to graduation, giving full countenance to exceptional circumstances as reported from the Exceptional Circumstances Committee and reassessment opportunities as detailed below.

MD 4.3 For Year 3, students must satisfy all progression requirements specified in MD4.2. For the written clinical science assessment, progression will be determined in accordance with the approved Year 3 assessment framework.

Students who satisfy the required written assessment standard will be deemed to have met the written assessment requirement. Where the approved Year 3 assessment framework identifies limited deficiencies in assessment evidence, students may be required to undertake a targeted reassessment.

Where the assessment profile demonstrates more extensive deficiencies, or where targeted reassessment is not permitted under the approved framework, the student will normally repeat Year 3. Progression following targeted reassessment requires successful completion of that reassessment together with satisfaction of all other Year 3 progression requirements.

MD 4.4 Students are required to pass each element of the course independently, to progress (i.e. compensation between elements is not allowed). Opportunities to resit a written paper, re-take a clinical exam, resit a Special Study Module or any other piece of coursework, or retake a clinical placement block in year 5 are offered where appropriate, and are signposted for students.

The Year 3 assessment framework does not permit compensation or condonation between assessment components. Targeted reassessment provides an opportunity for students to demonstrate achievement of specific learning outcomes where the approved progression framework identifies limited evidence deficits. Students must satisfy all other Year 3 progression requirements independently.

MD 4.5 Students have a chronological seven-year period following registration, within which they must complete their studies. This period includes time that may be taken out to repeat a year of study, complete an intercalated degree between years 4 and 5, or suspend studies for personal reasons. If exceptional circumstances mean that a

student will exceed this period, their case will be considered by the Head of the Medical School.

MD 4.6 Any student repeating a year must pass the resit year to progress. Students will not be permitted to repeat more than one year of study; if a student has previously resat a year, a repeat of a subsequent year is not permitted. In exceptional circumstances, the student's case will be considered by the Head of the Medical School.

MD 4.7 All students who graduate from academic year 2024-25 onwards must have passed the General Medical Council's Medical Licensing Assessment to be eligible to graduate with a Primary Medical Qualification. For information regarding the historic transitional agreement with the GMC, see appendix 2.

MD 4.8 To graduate with a Primary Medical Qualification, students must have passed both components of the Medical Licensing Assessment: the Applied Knowledge Test and the Clinical and Professional Skills Assessment. A pass in an MLA component will normally be expected to have been achieved within two years of the point of graduation. Where more than two years has elapsed, this will not automatically require the student to retake the component. The relevant Examination Board will consider whether there is sufficient current evidence that the student continues to meet the required standard for graduation and readiness for practice.

In making this decision, the Board will consider the student's subsequent engagement with, and performance in, the MBChB programme, including Year 5 workplace-based assessment, portfolio evidence, Professional Educational and Training Assessments, Preparing for Practice requirements, clinical placement performance, professionalism, fitness to practise information, and any other relevant assessment evidence. Where the Board is not satisfied that there is sufficient current evidence of readiness for practice, it may require the student to undertake further assessment before graduation. This may include repeating one or both MLA components, completing additional clinical placement activity, undertaking further workplace-based assessment, or repeating a period of study, depending on the circumstances of the case.

MD 5 CLASSIFICATION OF AWARDS

MD 5.1 The final award criteria for the MBChB degree have been detailed and approved through the programme's approval process. The year 5 Examination Board will determine whether a student has successfully met the final award criteria giving full countenance to exceptional circumstances as reported from the Exceptional Circumstances Committee, and reassessment opportunities as detailed below.

MD 5.2 In order to qualify for the overall award, students must have attained in full the minimum credit requirement for the programme by passing all the components of all years.

MD 5.3 There will be three classes of MBChB degree awarded, calculated based on the student's overall ranking in the cohort. Rankings are derived at the start of year 5 following the calculation of a weighted mean score (range 0 to 6.75) according to the formula shown below. Year 4 assessments receive more weight than those in years 1 to 3, as they are the final written and practical assessments.

MD 5.4 Formula for calculating an individual student's overall performance across the course:

$$score = \frac{6\left(\frac{\sum y_1 + y_2 + \cdots + y_n}{n}\right) + 3(x_4)}{4}$$

Where y_n denotes individual pieces of assessment in years 1-3 (these are all added together and then divided by the number of completed assessments) and x_4 denotes the mean score for year 4 (i.e. total points score over all assessments for year 4 ÷ number of assessments).

MD 5.5 The score for an individual assessment is calculated as follows.

(a) The standard-set pass mark for an individual written exam is adjusted to a 60% pass mark by arithmetic scaling. The new mark is then banded according to the following ranges:

80-100%	85% Band
70-79%	75% Band
60-69%	65% Band
50-59%	55% Band
Less than 50%	45% Band

(b) The percentage mark which the student has obtained in the OSCE is banded according to the following ranges:

> cut score + 2 standard deviations	85% Band
between >cut score + 1 s/d and cut score + 2 s/d	75% Band
between cut score and cut score + 1 s/d	65% Band
Failed (below cut score and/or failed on no. of stations)	55% Band

(c) Points are then assigned to the banded marks according to the following table:

Year of study	Written examinations, OSCEs	Coursework
1-4	Band 85 = 3 points Band 75 = 2 points Band 65 = 1 point Fail (first attempt) (bands 55 and 45) = 0 points	Excellent = 3 points Good = 2 points Pass = 1 point Fail (first attempt) = 0 points

MD 5.6 At the final Examination Board, the rankings (which are divided into deciles) are used to classify the awards as follows.

(a) Students in the 1st decile are awarded the degree of MBChB with Honours.

(b) Students in the 2nd decile are awarded the degree of MBChB with Commendation.

(c) All other students are awarded the degree of MBChB.

MD 6 REASSESSMENT

MD 6.1 A student who fails any component of the assessment in a given year (including SSM & coursework) will be required to undertake a reassessment for that component and must pass it before they can progress to the next year or graduate. All other coursework must be passed to graduate (see clauses MD 6.4-6.6 for further details of reassessment).

MD 6.2 Where permitted under the approved Year 3 assessment framework, reassessment may take the form of targeted reassessment. Targeted reassessment may comprise written assessment, OSCE, or both. Targeted reassessment constitutes the student's reassessment attempt unless undertaken as a replacement assessment following approved exceptional circumstances.

Students who do not satisfy the eligibility criteria for targeted reassessment will not normally be offered targeted reassessment.

MD 6.3 Students completing the Gateway Year who have not met the progression criteria outlined in MD.4.1 will be permitted to undertake reassessments to meet these criteria, as a second and final attempt.

MD 6.4 Students in the Gateway year and years 1-4 will be given the opportunity to undertake reassessment within the same academic year in which they made their first attempt. If a student fails one of the rotations of year 5, they will not be able to undertake reassessment within the same academic year but will be required to repeat some or all of the final year in the following academic year, subject to discussion of the case at the year 5 Examination Board.

MD 6.5 A student undertaking reassessment of a failed piece of coursework, including Special Study Modules (SSM), will be required to resubmit the coursework during the same academic year. The maximum mark permissible for the resubmitted coursework is "pass" (0 ranking points).

MD 6.6 When all the results of reassessment are available the overall profile will then be considered following procedures detailed in **Consideration and confirmation of results**.

MD 6.7 Exit awards and the criteria for exit awards are detailed in Appendix 3. Eligibility for an exit award is determined by successful completion of the relevant credit-bearing components, learning outcomes and assessment requirements. The Gateway year does not, in itself, count as completion of Year 1 of the MBChB programme for the purposes of determining MBChB exit award eligibility.

MD 7 INCOMPLETE ASSESSMENT AND EXCEPTIONAL CIRCUMSTANCES

MD 7.1 For the purposes of these regulations 'exceptional circumstances' will mean properly evidenced and approved claims from students that demonstrate good cause as to why their performance and achievements have been adversely affected by means which have not been fully addressed through extension and other available assessment procedures.

MD 7.2 For the purposes of these regulations 'good cause' will mean illness or other relevant personal circumstances affecting a student and resulting in either the student's failure to attend an examination, or submit coursework at or by the due time, or otherwise satisfy the requirements of the scheme of assessment of the MBChB programme; or, the student's performance in examination or other instrument of assessment being manifestly prejudiced.

MD 7.3 A chronic medical condition, for which due allowance has already been made, will not itself be considered a good cause although a short-term exacerbation of such a condition might be so judged.

MD 7.4 'Evidence' will mean a report descriptive of the medical condition or other adverse personal circumstances which are advanced by the student for consideration as amounting to good cause. Such a report should include a supporting statement from an appropriate person. Where the report refers to a medical condition of more than five days' duration the report must be completed by an appropriate medical practitioner who would be requested to comment on how the medical condition concerned would be likely (if this were the case) to have affected the student's ability to prepare for or carry out the assessment(s) in question.

MD 7.5 Where an incomplete assessment or impaired assessment performance may be the result of good cause, it will be the responsibility of the student concerned to make the circumstances known to Lancaster Medical School and to provide appropriate evidence.

Notification later than **48 hours after the end of a written examination or OSCE examination**, or later than **the published submission deadline for coursework or other submitted work**, will not normally be considered unless acceptable circumstances prevented the student from notifying the Medical School within this time.

MD 7.6 Lancaster Medical School will have an Exceptional Circumstances Committee whose primary responsibility it is to consider claims of good cause for the MBChB programme. Any such claims would be subject to confirmation by the Examination Boards later. The Exceptional Circumstances Committee would be required to meet at least once per annum prior to the final Examination Boards but might usefully meet to consider claims of good cause on a more frequent basis. The Exceptional Circumstances Committee will produce minutes of its meetings to be submitted to the appropriate Examination Board. Guidance on the management and operation of Exceptional Circumstances Committees can be found in General Regulations for Assessment and Award.

MD 7.7 In considering claims of good cause:

(a) the evidence provided by the student claiming good cause, and any relevant and available material submitted by them for assessment will be scrutinised.

(b) fairness to the individual student claiming good cause must be balanced with fairness to other students and the integrity of the assessment as a whole.

(c) in the event of the student having failed to attend an examination or examinations or having failed to submit course material or other work for assessment at or by the due time, it will be determined whether the failure to attend or submit has been justified by good cause.

(d) in the event of the student having submitted work for assessment by examination or otherwise, it will be determined whether such work has been manifestly prejudiced by good cause. If such prejudice is established the work affected will normally be deemed not to have been submitted.

MD 7.8 For MBChB students, where it is determined that the evidence presented supports their claim that their academic performance was affected by good cause during the academic year, or by circumstances which occurred during the summative examination period, the student will be automatically granted the right to take the resit examinations in August as if they were the first attempt, should they fail one or more components of the summative examinations. Should the student then fail the reassessment, they will be permitted to repeat the whole year of study. In this repeat year, they will sit all assessment components for that academic year in the summative exam period as if they were the second attempt.

MD 7.9 Where approved exceptional circumstances, assessment invalidation or other approved good cause means that a Year 3 assessment has not provided a valid assessment opportunity, the student will normally be offered a replacement assessment rather than targeted reassessment. A replacement assessment is intended to obtain the assessment evidence that could not validly be obtained at the original sitting.

Where a student has made a valid assessment attempt but has not demonstrated sufficient evidence for progression, any further assessment will normally constitute targeted reassessment or reassessment in accordance with the approved Year 3 assessment framework.

The Examination Board will determine whether an assessment constitutes a valid attempt, taking account of approved exceptional circumstances, assessment integrity and fairness to both the individual student and the wider cohort.

MD 7.10 For Gateway students, where it is determined that the evidence presented supports their claim that their academic performance was affected by good cause during the academic year, or by circumstances which occurred during the summative examination period, the student will be automatically granted the right to take the resit coursework or examinations as required in August as if they were the first attempt. If the student does not meet the progression requirements outlined in MD 4.1 after the reassessment period then they will be permitted to either:

i. resit any required assessments as an external candidate in the following academic year or
ii. repeat the year of study.

In the repeat year of study, students will be required to resit all assessments as if for the first time, with no marks from their previous year of study carried forward.

MD 7.11 Where it is determined that the evidence presented supports the student's claim that their academic performance was affected by circumstances which occurred during the resit examination period, the student will be automatically granted the right to repeat the whole year of study. In this repeat year, they will sit all assessment components for that academic year in the summative exam period as if they were the second attempt. If students suspend studies after the summative exams (and before resits) students will need to pass all assessment components for that academic year (this includes the OSCE examination, in appropriate years) to progress to the next academic year, to demonstrate continuity of knowledge and clinical skills.

MD 7.12 Where it is determined that the evidence presented supports the student's claim that their academic performance was affected by good cause during a period of coursework such as a four-week Special Study Module, meaning that they were not able to complete the coursework or were awarded a fail mark, they will be permitted to take a further coursework element during the summer which will be treated as the first attempt. Should this coursework be failed, another coursework element will be sat during the summer as the second attempt if there are sufficient weeks available to do this. If there is insufficient time, the student will be automatically granted the right for a further attempt at the coursework element but will suspend studies rather than repeat the year; it is not possible for the student to progress into the next year until the coursework element has been passed. A student in this position will not be required to resit the OSCE examination.

MD 7.13 With the exception of year 4, students who have not passed all assessments following the annual resit examination period will have their studies terminated with the right to appeal. If an appeal is successful, the only option for MBChB registered students is to repeat the whole year of study. It is not possible for any failed components to be carried into the next year of study.

MD 7.14 Students who have not passed all Year 4 assessments after the resit examination period have an automatic right to repeat the whole year of study once. In a repeat year, students must sit and pass all Year 4 assessment components in that academic year, including the Medical Licensing Assessment Applied Knowledge Test, the Medical Licensing Assessment Clinical and Professional Skills Assessment, and the required portfolio and placement components.

No Year 4 assessment component may be carried forward from a previous academic year for the purposes of progression to Year 5, unless this is required by external regulatory requirements or approved through formal University processes.

Students may normally have up to four opportunities to pass the Year 4 MLA-AKT and CPSA: the summative and resit opportunities in the original Year 4, and the summative and resit opportunities in any approved repeat Year 4. This does not create an

entitlement to four assessment sittings, nor does it create an entitlement to more than one repeat year.

Where a student misses an assessment, is unable to sit, is deemed not to have made a valid attempt, or has approved exceptional circumstances affecting an assessment opportunity, the relevant Board will determine whether the affected opportunity is counted as an attempt in accordance with the exceptional circumstances regulations. Approval of exceptional circumstances may affect the status of an individual attempt but does not automatically grant an additional repeat year or further assessment opportunity outside the approved structure of the programme.

Students who have repeated Year 4 and exhausted the assessment opportunities available within that repeat year without passing all required Year 4 assessment components will have their studies terminated with the right to appeal. Any further opportunity following a successful appeal will be exceptional and subject to approval through the relevant University process.

MD 7.15 Where it is determined that the evidence presented does not support the student's claim that their academic performance was affected by good cause during the academic year, or by circumstances which occurred during the examination periods, or during a period of coursework, the student's original assessment result will stand and they will have to resit any failed assessment components as the second and final attempt for the year. With the exception of year 4 (see clause 7.12), where a student fails at the second attempt resit stage, their studies will be terminated. Students in year 4 will have their studies terminated after the resit stage of their repeat year.

MD 7.16 A student who is permitted by an Academic Appeals Panel to repeat the year will have to sit all assessments but will be granted the same number of attempts as the rest of the cohort i.e. will be permitted the August resit opportunity. For the purposes of ranking, the student will receive the points relating to their <u>first</u> attempt:

- 0 points for the failed components
- The first attempt marks for those components which were passed.

When taking previously passed components in the repeat year, students must pass each component again to progress to the next year. However, as stated above, they will receive ranking points relating to their first attempt, hence it is not possible to gain higher points in the repeat year.

MD 8 CONSIDERATION AND CONFIRMATION OF RESULTS

MD 8.1 Senate has ultimate authority to determine all results of assessment leading to Lancaster University credit and awards. It exercises its authority to make final decisions as to granting of all credit-bearing University awards, primarily through the Committee of Senate with non-standard cases considered and recommended by the Classification and Assessment Review Board.

MD 8.2 The Committee of Senate provides:

(a) formal confirmation (or not) of recommendations from Boards of Examiners for the award to individual students of the MBChB degree of a particular class.

(b) formal approval of recommendations from Boards of Examiners that students be awarded no degree with or without a further re-sit opportunity (i.e. Fails).

(c) formal ratification of first, second, third, fourth-, and fifth-year results in the MBChB degree, including the timing and nature of re-sit opportunities for failed elements.

Further procedural details are set out in the General Regulations for Assessment and Award.

MD 8.3 For each year of the MBChB programme there will be an Examination Board comprising external and internal examiners which will be responsible for the assurance of standards through the exercise of their academic (and, where relevant, clinical/practitioner-based) judgement both directly in the assessment of students' work and indirectly in the design of specific forms of assessment. Clinicians who are nominated by Lancaster Medical School because they have primary responsibility for part of the medical degree programme including assessment will be entitled to be members of Examination Boards and will be termed “internal examiners”. The constitution and terms of reference for examination bodies within the constituent elements of the University are set out in the section on Examination Boards in the General Regulations for Assessment and Award.

MD 8.4 The Examination Boards will receive decisions from the Exceptional Circumstances Committee. Examination Boards cannot, of themselves, reconsider or change decisions of the Exceptional Circumstances Committee. Examination Boards may challenge decisions of Exceptional Circumstances Committees by referring final decisions to the Committee of Senate via the Classification and Assessment Review Board, or equivalent body.

MD 8.5 The Examination Boards for years 1-4 will consider and confirm marks derived from all assessments taken and examined in the academic year under consideration. The year 5 Examination Board will receive the results of assessments from years 1-4 and will consider and confirm the year 5 assessments. The year 5 Examination Board will make recommendations to the Committee of Senate with non-standard cases referred for consideration and recommendation via the Classification and Assessment Review Board as to the award of the MBChB degree (and the class of degree) within the approved degree programme classification scheme. Details of the role and operation of Examination Boards can be found in the General Regulations for Assessment & Award.

MD 8.6 The business of the Examination Boards will be minuted and the minutes will include a record of the External Examiner's adjudications, comments and recommendations, as well as decisions made by the Board. The minutes will also record the decisions of the Exceptional Circumstances Committee for each candidate considered by that committee (although detailed discussion of circumstances should not be undertaken at the Examination Board). The minutes must include a list of attendees (together with their status as external or internal examiner or assessor). This record of the proceedings of the Board will be restricted and made available only to: the participating examiners and assessors, the Vice-Chancellor and other officers of the University as appropriate; the Committee of Senate and the Classification and Assessment Review Board; and appropriate Academic Appeal and Review Panels as

defined in the chapter on Academic Appeals. Where the Examination Board has exercised its discretion in a particular case, as provided by these Regulations, the Committee of Senate via the Classification and Assessment Review Board will normally uphold its decision providing it had majority support of the external examiners present at that Examination Board.

MD 9 PUBLISHED INFORMATION

MD 9.1 The determination of results and the classification of university degrees are subject always to ratification by the Committee of Senate and will be regarded as provisional until ratified.

MD 9.2 Immediately after the meeting of the relevant Examination Board, Lancaster Medical School may notify students of their provisional degree results.

MD 9.3 Within forty days of the ratification of degree results, students will be sent a transcript of their results together with a Higher Education Achievement Report, both of which will conform in scope and layout to principles agreed by Senate.

MD 10 EXCLUSION

MD 10.1 Students who, after undertaking agreed reassessment opportunities, fail to meet the stipulated criteria for progression or final award will be excluded from the University. Students are entitled to appeal against exclusion under the University's Academic Appeals procedures.

APPENDIX 1: HISTORIC PROGRESSION REQUIRMENTS

2016-18 ***For students who entered the programme in 2016-18*:**

Practical examinations (the Objective Structured Clinical Examinations, (OSCEs) in years 1-4): each OSCE was standard set using a borderline regression approach. The marks of all students were used to determine the passing score for each station, and to pass overall, students were required to achieve the "cut score", which was the sum of the pass marks for each station in that examination. Students were also required to pass a minimum of 64% of stations in the OSCEs in years 1 and 2 and 71% of stations in years 3 and 4.

2018-2019 ***For students who entered the programme from 2018-19:***

Practical examinations (the Objective Structured Clinical Examinations, (OSCEs) in years 1-4): each OSCE was standard set using a borderline regression approach. The marks of all students were used to determine the passing score for each station, and to pass overall, students were required to achieve the "cut score", which was the sum of the pass marks for each station in that examination. Students were also required to pass a minimum of 70% of stations (representing 7 out of 10 stations) in the OSCE in year 1 and 71% of stations in years 2, 3 and 4 (representing 10 out of 14 stations).

2019-2020 ***For students who entered the programme from 2019-20:***

Practical examinations (the Objective Structured Clinical Examinations, (OSCEs) in years 1-4): each OSCE was standard set using a borderline regression approach. The marks of all students were used to determine the passing score for each station, and to pass overall, students were required to achieve the "cut score", which was the sum of the pass marks for each station in that examination. Students were also required to pass a minimum of 62.5% of stations (representing 5 out of 8 stations) in the OSCE in year 1 and 71% of stations in years 2, 3 and 4 (representing 10 out of 14 stations).

2020-2021 ***For students who entered the programme from 2020-21 onwards:***

Practical examinations (the Objective Structured Clinical Examinations, (OSCEs) in years 1-4): each OSCE was standard set using a borderline regression approach. The marks of all students were used to determine the passing score for each station, and to pass overall, students were required to achieve the "cut score", which was the sum of the pass marks for each station in that examination. Students were also required to pass a minimum of 71% of stations in the OSCE in years 2, 3 and 4 (representing 10 out of 14 stations).

Note: *During the academic year 2020-21, the number of OSCE stations was reduced for students in years 2 and 3 in mitigation of the COVID-19 pandemic.*

During the academic year 2021-2022, the number of OSCE stations was reduced for students in year 2 in mitigation of the ongoing impact of the COVID-19 pandemic. During the academic year 2022-23, for the Year 4 Summative OSCE only, the number of stations was reduced to 10 stations (students had to pass 7 out of 10 stations) due to the impact of Junior Doctors' strike action on the delivery of this assessment. The Year 4 Resit OSCE remained at 14 stations.

During the academic year 2024-2025, the number of OSCE stations was reduced for students in year 2 for the Resit OSCE to 12 stations (students had to pass 8 out of 12 stations) due to the impact of Junior Doctors' strike action on the delivery of this assessment. The Year 3 and Year 4 Resit OSCE remained at 14 stations.

APPENDIX 2: HISTORIC TRANSITIONAL AGREEMENT FOR THE MLA

Exceptionally, under the transitional arrangements agreed by the General Medical Council (GMC), any student who has successfully passed all written and clinical finals assessments in their penultimate year of study and is out of programme in the academic year 2023-24 will not need to take an Applied Knowledge Test (AKT) or Clinical and Professional Skills Assessment (CPSA) that the GMC has confirmed meets the requirements of the Medical Licensing Assessment Framework[1] on return to programme if they meet the following criteria set by the GMC:

a. The student is out of programme in the academic year 2023-24 with the medical school's approval and

   i. In their penultimate year of study in 2022-23 has taken and passed the written and clinical exams as part of a degree programme currently recognised by the GMC.

   ii. Has met all the other academic requirements of the university for progression to the final year of study and

   iii. Must have graduated in or before the end of the academic year 2025-26

b. If any of the above criteria are not met, the student must pass an AKT and a CPSA that the GMC has determined meets the requirements of the MLA Framework prior to graduation.

[1] Assuring readiness for practice: a framework for the MLA. mla-framework-plus-annexes-pdf-85707459_pdf-88217761.pdf (gmc-uk.org)

APPENDIX 3: EXIT AWARDS FOR THE MBCHB PROGRAMME

Students ineligible for a Primary Medical Qualification may qualify for an exit award. Exit award criteria are detailed below.

Exit award eligibility will be determined by the relevant Examination Board based on the student's successful completion of the required credit-bearing MBChB curriculum components, assessment components and attainment of the programme learning outcomes for the relevant stage of study. Exit awards may be considered where any of the following apply:

- A student does not progress to the next or final stage of the MBChB programme because of academic performance;
- A student chooses not to continue their studies;
- A student is no longer enrolled on the MBChB programme, or eligible to graduate on the MBChB programme following a Fitness to Practise or other disciplinary procedure

For students on the (A104) Gateway programme, successful completion of the Gateway year permits progression onto the rest of the MBChB programme but does not constitute completion of Year 1 of the (A100) MBChB programme for the purposes of exit award eligibility. For the purposes of exit awards, stage 1 is the equivalent of year 1 for the A100 MBChB or year 2 (year immediately following Gateway year) for the A104 MBChB with Gateway. Stage of study increments with each subsequent year of study.

Where assessment within the MBChB programme is organised by clinical rotation, assessment window, longitudinal theme, programme stage, or other curriculum structure, completion of the relevant stage for the purposes of an exit award requires successful completion of ***all*** assessment components, attainment of all learning outcomes and achievement of all progression requirements assigned to that stage. This may include rotation-specific assessments, longitudinal assessments, clinical skills assessments, coursework, portfolio evidence, professionalism requirements and any required reassessment.

Criteria for specific exit awards, in addition to those detailed above, are as follows:

1. BSc (Hons) Medical Sciences may be awarded to a student who leaves the MBChB programme before completion of the full MBChB, provided that the student has successfully completed all required MBChB curriculum components, assessment components and attained programme learning outcomes equivalent to completion of the first three stages of the MBChB programme.

   This would normally require successful completion of all required assessment components associated with MBChB stages 1, 2 and 3, including written assessments, clinical skills / OSCE assessments, coursework, portfolio or professionalism requirements, and any other progression requirements specified for those stages of the programme.

2. BSc (Hons) Advanced Medical Sciences may be awarded to a student who leaves the MBChB programme before completion of the full MBChB, provided that the student has successfully completed all required MBChB curriculum components, assessment components and programme learning outcomes equivalent to completion of the first four stages of the MBChB programme.

For the current curriculum, this would normally require successful completion of all required assessment components associated with MBChB stages 1, 2, 3 and 4, including written assessments, clinical skills / OSCE assessments, coursework, portfolio or professionalism requirements, and any other progression requirements specified for those stages of the programme.

Where stage 4 includes Medical Licensing Assessment components, the student must have satisfied the assessment requirements for that stage of the programme, but award of the BSc (Hons) Advanced Medical Sciences does not constitute award of a Primary Medical Qualification and does not confer eligibility for provisional registration.

This award may be considered where a student does not successfully complete stage 5.

Classification of Exit Awards

Classification of exit awards will be determined using the approved MBChB classification methodology set out in MD 5.4–MD 5.6, or any successor provisions. Only marks from assessment components that contribute to the relevant exit award classification will be included. Where assessment structures have changed, the Examination Board will apply the approved mapping or equivalence rules to ensure comparability between cohorts.

The BSc (Hons) degree will be classified according to the decile as:

Deciles 1 and 2 – first class

Deciles 3, 4, 5, 6 and 7 – upper second class

Deciles 8 and 9 – lower second class

Decile 10 – third class

APPENDIX 4: SUMMARY OF CHANGES FROM 2025/26 MARP

This appendix summarises the principal amendments made to the Medical Degree Assessment Regulations for 2026/27. It is intended to explain how the regulations have changed and why. Minor typographical, formatting and consequential numbering changes are not individually listed unless they affect the interpretation of the regulations.

Clause / area	2025/26 position	2026/27 position	Reason for change
MD1.4 – Attendance and engagement	Attendance and active engagement in scheduled teaching, clinical placements and prescribed activities were mandatory. Students were required to demonstrate satisfactory attendance and engagement as determined by Medical School monitoring.	Retains attendance and engagement as mandatory requirements, but refers to the **published attendance, engagement and professional requirements** for each year and the Medical School's approved monitoring processes.	Clarifies the regulatory requirement while allowing detailed operational expectations and monitoring arrangements to be published in programme guidance rather than duplicated in MARP.
MD1.5 – Inclusive assessment and reasonable adjustments	Lancaster Medical School written examinations included 25% additional time for all students. The interaction between this universal adjustment and an individual ILSP was not explicit.	Clarifies that the published examination duration already includes the universal 25% adjustment and that an ILSP does not automatically add a further 25%. Additional reasonable adjustments continue to be considered individually. It also clarifies that external assessments follow the relevant provider's arrangements.	Removes potential ambiguity about cumulative additional time and distinguishes LMS assessments from externally regulated assessments.
MD1.6 – Programme documentation	Referred specifically to the Courses Handbook.	Refers more generally to the relevant programme documentation.	The former named handbook is no longer the sole location of programme information.
MD2.1 – Standard-setting reference	Included a specific reference to the 2015 GMC publication *Promoting Excellence*.	States more generally that standard-setting procedures are undertaken in accordance with GMC requirements.	Removes an outdated publication reference while retaining the regulatory principle.
MD2.2(a) – Written examination standard setting	Written papers were usually standard set by content experts, usually using modified Angoff.	Streamlines the wording while retaining an agreed standard-setting method, normally modified Angoff, and subsequent review using relevant assessment evidence.	Clarifies existing practice; no substantive change to standard setting.
MD2.2(b) – Year 3 written assessment	No equivalent Year 3-specific provision.	Introduces the revised Year 3 written assessment structure across two assessment	Implements the redesigned Year 3 assessment framework

		windows and clinical science domains. Outcomes may be classified as passed, borderline or significant evidence deficit, with borderline defined using two standard errors of measurement.	and provides transparent rules for interpreting domain-level performance.
MD2.2(c) – OSCE standard	The minimum station requirement was described as 71%, representing 10/14 stations.	Retains the same requirement but expresses it directly as 10/14 where a standard 14-station OSCE is used.	Improves clarity; no change in standard.
MD2.2(d) – Year 3 OSCE	No equivalent Year 3-specific provision.	Introduces the two-window Year 3 OSCE model, combined aggregate and station-count requirements, seven-station targeted reassessment where appropriate, and the distinction between targeted and replacement assessment.	Implements the redesigned Year 3 assessment and reassessment model.
MD4.2 – Progression requirements	Year 3 progression required Papers 1 and 2, HCS and PPVE coursework, OSCE, placement attendance and completion of the ePortfolio.	Year 3 progression is updated to reflect clinical science assessment across the new assessment windows and domains, the combined OSCE profile, Medical Ethics and Law, Epidemiology and Evidence-Based Medicine, workplace/portfolio requirements and placement attendance. Common attendance and engagement requirements are also referenced consistently through MD1.4.	Implements the approved redesigned Year 3 curriculum and improves consistency across progression requirements.
MD4.3 – Year 3 progression following assessment	No separate Year 3 progression provision.	Establishes that all MD4.2 requirements must be satisfied. Limited deficiencies may lead to targeted reassessment, while more extensive deficiencies will normally require repetition of Year 3.	Provides clear regulatory rules for applying the new Year 3 progression framework.
MD4.4 – Compensation and targeted reassessment	The regulations already prohibited compensation between assessment elements.	Retains that principle and explicitly confirms that Year 3 targeted reassessment does not constitute compensation or condonation.	Clarifies how the new Year 3 evidence-based progression model relates to the existing prohibition on compensation.

MD4.7 / Appendix 2 – Historic MLA transitional agreement	The detailed GMC transitional agreement appeared in the main progression regulations.	The operative MLA requirement remains in MD4.7, while the historic transitional agreement is retained separately in Appendix 2.	Keeps current regulations concise while preserving historic provisions for any students to whom they remain relevant.
MD4.8 – MLA timing and readiness for practice	Students graduating more than two years after passing the MLA components were required automatically to sit and pass both again.	A pass within two years remains the normal expectation, but more than two years does not automatically require a retake. The Examination Board instead considers current evidence of readiness for practice and may require further assessment where necessary.	Reflects the previously agreed approach to MLA validity, allowing proportionate consideration of contemporary evidence while preserving patient-safety and readiness-for-practice requirements.
MD6.2 – Targeted reassessment in Year 3	No equivalent provision.	Introduces targeted reassessment for eligible Year 3 students. This may comprise written assessment, OSCE, or both, and is distinguished from replacement assessment following accepted exceptional circumstances.	Implements the approved Year 3 reassessment model.
MD6.7 / Appendix 3 – Exit awards	Detailed BSc (Hons) Medical Sciences and BSc (Hons) Advanced Medical Sciences eligibility and classification provisions appeared within MD6.7.	MD6.7 now provides the overarching principle and directs readers to **Appendix 3**, which contains the detailed exit-award requirements and classification rules.	Moves detailed exit-award regulations into a dedicated appendix, improving readability and allowing them to accommodate changes in curriculum structure more readily.
MD7.5 – Notification of exceptional circumstances	Required notification within 48 hours “after the examination” or after coursework was due.	Clarifies that the period runs from the end of a written examination or OSCE, or the published submission deadline for coursework or other submitted work.	Removes ambiguity about when the 48-hour notification period begins.
MD7.9 – Year 3 replacement assessment	No Year 3-specific equivalent provision. The previous MD7.9 concerned Gateway students.	Introduces a specific distinction between an assessment opportunity that is invalid because of accepted exceptional circumstances/invalidation and a valid attempt that produces insufficient evidence. Invalid opportunities normally lead to replacement assessment; valid attempts may lead to targeted reassessment.	Ensures students who did not have a valid opportunity to provide evidence are not treated as having failed a valid assessment attempt.

MD7.14 – Repeat Year 4	Students repeating Year 4 did not need to retake a previously passed AKT, although a previously passed OSCE had to be passed again.	Requires all Year 4 components to be passed within the repeat academic year and clarifies the available assessment opportunities, treatment of invalid attempts and consequences of exhausting those opportunities.	Reflects the previously agreed Year 4 repeat-year arrangements and clarifies how assessment opportunities operate.
Consequential renumbering and appendices	Historic MLA arrangements and exit awards were embedded within the main regulations, and MD7 numbering followed the previous sequence.	Historic MLA arrangements are in Appendix 2; exit awards are in Appendix 3; this summary becomes Appendix 4. Introduction of the new MD7.9 results in consequential renumbering of later MD7 clauses.	Improves document structure and ensures cross-references remain accurate.

APPENDIX 5: DETAIL OF IN-YEAR CHANGE MADE TO CHAPTER

Version	Date of change	Detail of change & sections altered	Approval of change
1.1	01/10/2026	4.1: clarification of progression criteria from Gateway to MBChB, to agree with module design (BIOS4101 no longer has an end-of-year examination).	MBChB Assessment Committee, 30/09/26